

CAPITAL MARKETS

Brookfield's \$1B Medical Office Bet: Capital for Control, Not Just Yield

The JV gives Healthpeak long-term capital without losing operating control, revealing a market where liquidity comes with strings attached.

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A source-grounded Light Tower Insight. The complete note follows.

The headline reads like a straightforward capital infusion: Brookfield Asset Management buys a 49% stake in a \$2.1 billion medical office portfolio from Healthpeak Properties. But the more revealing number is not the \$1 billion Brookfield contributed. It is the 6.5% net annual return Healthpeak must deliver to buy Brookfield out after seven years.

That call right is the economic center of this transaction. It tells you that Brookfield is not simply a passive capital partner. It is a patient investor with a defined exit timeline and a guaranteed floor return. Healthpeak, in turn, is not selling assets. It is selling a slice of the cash flow to fund its balance sheet while retaining operating control and the option to repurchase the stake at a known cost.

This is a market where liquidity is available, but only under terms that protect the capital provider from downside. The days of cheap, unsecured corporate debt are over. The capital that is moving now comes with guardrails.

The portfolio itself is 86 properties totaling 5.6 million square feet across 11 states, 95% leased. That occupancy rate is the kind of stabilized cash flow that institutional capital can underwrite. It is not speculative. It is not development risk. It is a bond-like income stream attached to real estate with a secular demand driver: outpatient care.

Healthpeak CEO Scott Brinker framed the deal as advancing the REIT's capital allocation priorities while capturing tailwinds in outpatient care. That is the public narrative. The private one is more specific: Healthpeak needed long-term capital without triggering a sale that would crystallize a discount to net asset value. Its shares have risen nearly 40% this year, but that still leaves the cost of equity capital above the cost of a structured joint venture. By selling a minority stake, Healthpeak avoids the dilution of a secondary offering and the loss of control that comes with a portfolio sale.

Brookfield, meanwhile, is deploying capital from a record \$112 billion raised in 2025, including \$13.3 billion in real estate commitments. It also raised approximately \$900 million for its second fund targeting recapitalizations and equity solutions. This deal fits squarely in that mandate: provide capital to a high-quality owner that needs balance sheet relief, but structure the investment so that Brookfield gets a preferred return and a clear path to liquidity.

The 6.5% net annual return is the key term. It is not a market-clearing yield for a 95% leased medical office portfolio in a low-leverage structure. It is a negotiated rate that reflects the cost of patience. Healthpeak is paying for the right to wait seven years before buying Brookfield out. If interest rates fall in that period, the 6.5% will look expensive. If rates stay elevated, it will look like a fair price for certainty.

This transaction also reveals something about the broader market for healthcare real estate. Medical office is not office. It is a distinct asset class with different demand drivers, tenant credit profiles, and capital structures. The 95% occupancy rate is not a cyclical peak. It is a structural feature of a sector where tenants are health systems and physician groups with long-term leases and essential services. That is why Brookfield is willing to write a \$1 billion check at a time when most institutional capital is still sitting on the sidelines for traditional office.

But the deal is not a signal that medical office is immune to the broader repricing of real estate. It is a signal that capital is available for assets with defensible cash flow and sponsors with balance-sheet credibility. Healthpeak has both. The 86-property portfolio is diversified across 11 states. The sponsor is a publicly traded REIT with a focused strategy after spinning off its senior housing assets in March. The operating platform is intact. Brookfield is not underwriting a turnaround. It is underwriting a continuation.

The practical implication for other owners is straightforward. If you have a stabilized portfolio in a sector with secular demand, you can access institutional capital. But the terms will reflect the capital provider's need for downside protection. Expect preferred returns, call rights, and defined exit timelines. Expect the capital to come with strings attached.

For lenders, this deal is a reminder that the equity layer is becoming more structured. Joint ventures with preferred returns and buyout options are not new, but they are becoming more common as traditional debt markets remain selective. The equity that is available is not passive. It is active, patient, and priced for protection.

The question the market should test next is whether this structure can scale beyond healthcare real

estate. Can a similar joint venture work for industrial, multifamily, or even well-located office? The answer depends on whether the cash flow is stable enough to underwrite a 6.5% preferred return and whether the sponsor is credible enough to execute the buyout. That is a narrow set of conditions. But it is wider than it was a year ago.

Brookfield and Healthpeak have given the market a template. The next deal will tell us whether it is a one-off or a pattern.

SOURCES

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<https://www.bisnow.com/national/news/healthcare/brookfield-healthpeak-form-21b-medical-office-portfolio-jv-135482>

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